

NMHS Hospital Management Information System

User Manual & Step-by-Step
Hospital Workflow Guide

SYSTEM	NMHS-HMIS-V1.0
DOCUMENT	NMHS-HMIS-V1.0-UM
VERSION	1.0
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REPOSITORY INSPECTED	main @ 1da43f3 + uncommitted Procedure work

Source of truth. This manual describes the NMHS-HMIS-V1.0 application as it exists in the repository inspected on 1 October 2026 (branch `main`, commit `1da43f3`, plus the uncommitted working tree that separates the Procedure department from Theatre). Screens, menus, permissions and workflows were read from the application's own code. If the software is updated, sections of this manual may need to be revised.

Contents

1. Introduction	5
2. What the HMIS does	5
3. Who should use this manual	5
4. User roles	6
5. Signing in and out	7
5.1 Signing in	7
5.2 Wrong password and the five-attempt lockout	7
5.3 Signing out and shared computers	7
5.4 Passwords	7
6. Permissions and department access	7
6.1 Where department membership matters	8
6.2 Examples	8
7. Basic navigation	8
7.1 Menu items by role	8
8. Dashboard overview	9
9. Front desk: patients and reception	10
9.1 Key ideas	10
9.2 Registering a new patient	10
9.3 Finding a returning patient	10
9.4 Billing a card or consultation fee	10
9.5 Routing a patient (the queue)	11
9.6 Patient record navigation	12
10. Appointments	12
10.1 Queuing an appointment	12
10.2 What happens next	12
10.3 Configuring what can be booked (administrators)	12
11. Nursing and triage	13
11.1 Taking vitals	13
11.2 Handing the patient to a doctor	13
12. Doctor workflow	13
12.1 Starting the consultation	14
12.2 Writing the medical note	14
12.3 Health record	14
12.4 Referring to a department	14
12.5 Prescribing	15
12.6 Completing the consultation	15

13. Laboratory	15
13.1 Receiving and working a request	15
13.2 Payment	15
13.3 Laboratory catalogue	16
13.4 Where results appear	16
14. Ultrasound / Imaging and Eye Clinic	16
14.1 Ultrasound / Imaging	16
14.2 Eye Clinic	16
15. Pharmacy	16
15.1 Two different workflows	16
15.2 Dispensing a prescription	17
15.3 Walk-in sales on the POS	17
16. Inventory	18
16.1 Concepts	18
16.2 Who does what	18
16.3 Receiving stock	18
16.4 Transferring to the Pharmacy	19
16.5 Stock counts	19
16.6 Expired items	19
16.7 Movement history	19
17. Billing, cashier and finance	19
17.1 Where charges come from	19
17.2 Taking a payment	20
17.3 Who can do what	20
17.4 Part payments, pay later, discounts and waivers	20
17.5 Refunds and service cancellations	20
17.6 Registers and reports	21
18. Maternity	21
18.1 At the maternity desk (reception)	21
18.2 Pregnancy and ANC	21
18.3 The maternity workspace	22
18.4 Maternity billing	22
18.5 Maternity documents	22
19. Procedure Department	22
19.1 Who are Procedure staff	23
19.2 Ordering a procedure (doctor)	23
19.3 Performing and documenting (Procedure staff)	23
19.4 After completion	23

20. Theatre	24
21. Inpatient and emergency	24
21.1 Admissions (bed board)	24
21.2 Admin discharge workspace	24
21.3 Emergency	24
22. The patient chart and history	25
23. Printing	25
24. Administration	26
24.1 Staff accounts (Users)	26
24.2 Departments and staff membership	26
24.3 Configuration pages	26
25. Security and good practice	27
26. Troubleshooting	27
27. Quick reference	28
27.1 Role → main responsibilities	28
27.2 Department → main workflow	28
27.3 Terminology	28
27.4 Support and escalation	29

1. Introduction

The NMHS Hospital Management Information System (**NMHS-HMIS-V1.0**) is the software Ngozi Maternity and Hospital Services uses to register patients, move them between departments, record their care, bill for services, take payments, dispense medicine and keep stock. It runs in a web browser on a desktop, laptop, tablet or phone.

This manual explains, step by step, how each member of staff does their part of the patient's journey: what to open, what to click, what to type, what happens next, who receives the work, what money is involved and what can be printed.

NOTE: How to read the steps

Menu names are written exactly as they appear in the left-hand menu, for example **Billing** or **Refer Patient**. Buttons are written in bold, for example **Accept**. Your menu only shows what your role is allowed to use, so you may not see every item mentioned in this manual.

2. What the HMIS does

Area	What the HMIS provides
Front desk	Patient registration with a hospital number, patient card printing, routing patients to departments, the appointment queue.
Clinical	Triage vitals and nursing notes, doctor consultations and medical notes, the nine-section health record, referrals, prescriptions, the full patient chart.
Departments	Laboratory, Ultrasound / Imaging, Eye Clinic and Procedure stations, each with its own queue, result recording and printed report.
Maternity	Pregnancy episodes, antenatal (ANC) visits, labour and partogram, delivery, newborns (including twins and other multiple births) and postpartum care.
Inpatient	Wards and beds, admission, bed transfer and discharge with a discharge letter.
Pharmacy	Prescription queue, per-line billing, dispensing (FEFO), and a walk-in Point of Sale (POS) till with discounts, returns and receipts.
Inventory	Product catalogue, batches with expiry dates, Main Store and Pharmacy locations, receiving, transfers, stock counts, expired items, movement log.
Billing & finance	Charges, payments, part payments, pay later, discounts, waivers, refunds, service cancellations, outstanding balances, statements and the financial report by department.
Administration	Users, roles, departments and their staff, services and prices, wards and beds, hospital settings, notification settings.

3. Who should use this manual

- **Front desk / reception** — registration, routing, appointments and billing at the window.
- **Nurses and maternity nurses (midwives)** — triage, vitals, nursing notes, maternity care.
- **Doctors and eye doctors** — consultations, referrals, prescriptions, the patient chart.
- **Laboratory, radiology, optometry and Procedure staff** — department stations.
- **Pharmacists** — dispensing, the pharmacy shelf and the POS till.
- **Cashiers and accountants** — payments, discounts, waivers, refunds, cancellations, reports.
- **Inventory managers** — the Main Store, receiving, transfers, counts and expired stock.
- **Administrators** — staff accounts, departments, prices, wards, beds and settings.

Technical installation, hosting, backups and server administration are covered in the separate **Deployment, Hosting & System Administration Guide**.

4. User roles

Every staff account has exactly **one role**. The role decides which menus you see and which actions the system accepts from you. The roles below are the ones the system defines; the name in the first column is how the role appears on the Users screen.

Role	Main work in the HMIS
Super Admin	Everything. Only the Super Admin may delete a patient registration (and only one with no history).
Hospital Admin	Everything an administrator does day to day: users, departments, configuration, inventory catalogue, admin discharge. Passes every role check except the Super-Admin-only actions.
Doctor	Consultations, medical notes, health record, referrals, prescriptions, appointments, wards. Procedure work only if posted to the Procedure department.
Ophthalmologist / Eye Doctor	The Eye Clinic station plus the doctor's chart, notes (with the eye examination), prescriptions and referrals — for the patients they hold.
Nurse	Triage (vitals and nursing notes), Send to Doctor, wards. Procedure work only if posted to the Procedure department.
Maternity (maternity nurse / midwife)	The Maternity workspace: pregnancies, ANC, labour, delivery, newborns, postpartum, maternity triage.
Reception	Registration, routing, appointments, billing at the window, pay-later authorisations, maternity lookup and assignment.
Pharmacist	Prescription queue, dispensing, taking payment for pharmacy bills, the POS till (without discounts), counting the pharmacy shelf.
Laboratory Scientist	Laboratory station, results, verification, laboratory catalogue.
Radiology Staff	Ultrasound / Imaging station and imaging reports.
Optometrist	Eye Clinic station.
Billing Officer / Cashier	Payments, discounts, waivers, refunds, cancellations, the billing catalogue, POS till (with discounts and returns), financial report.
Accountant	As the cashier for money matters, plus approving POS discounts above the cashier's limit. No POS returns.
Inventory Manager	Inventory desk: receiving, transfers, applying counts, write-offs, expired items.
Ward Manager	Patients list and the Admissions bed board.
Surgeon, Anesthetist, Records Officer, Management / Executive, Custom role	These roles exist on the role list but the current system gives them no dedicated workspace : they see the dashboard and their notifications only.

IMPORTANT: A role is not a department

Your **role** says what kind of work you do (for example Nurse). Your **department membership** says where you are authorised to do it (for example Procedure or Maternity). Some screens need both — see section 6.

5. Signing in and out

5.1 Signing in

1. Open the HMIS address your administrator gave you in a web browser. You will see the **Sign in** page.
2. Type your **username** and **password** and press **Sign in**.
3. On success you arrive at your **Dashboard**. Your name and role are shown at the top right.

5.2 Wrong password and the five-attempt logout

- A wrong username or password shows *Invalid username or password*. The message is the same whether or not the username exists.
- Four failed attempts in a row are allowed. The **fifth** locks that username **from that computer's network address** for **5 minutes** (the default; your administrator can change it). A countdown is shown and the form unlocks itself when the time is up.
- While locked, even the correct password is refused. A successful sign-in clears the count.
- The lockout never disables your account permanently. If you have forgotten your password, ask an administrator to set a new one.

5.3 Signing out and shared computers

Press **Sign out** at the top right. Your sign-in is ended on the server and the screen is cleared, so the next person at a shared computer cannot see your notifications or your work. If your session ends for any reason, the HMIS returns you to the Sign in page.

5.4 Passwords

- Staff cannot change their own password inside the HMIS. An administrator sets or resets it from **Users** (or Django Admin's **Set password** link).
- Administrators can tick *must change password* on an account as a reminder; the current system records this flag but does **not** force a change at sign-in.
- Never share your password: every record you save is stamped with your name.

6. Permissions and department access

Access in the HMIS is decided by the **server**, not by the screen. Menus are hidden only as a courtesy so you are not offered something that would be refused. Two things decide what you can do:

	Role	Department membership
What it is	One per account, chosen by an administrator.	Zero or more departments; an administrator adds you to a department's Staff list.
What it decides	The kind of work you may do: prescribe, take vitals, dispense, refund...	Where some of that work applies: the Procedure department, Maternity's team.
Can you have several?	No — one role.	Yes — any number of departments at once.

6.1 Where department membership matters

Workspace	Who may use it
Procedure	A Doctor or Nurse who is on the Procedure department's staff, plus administrators. Being on Theatre's staff gives nothing here.
Maternity (as a doctor or nurse)	Doctors and nurses on the Maternity department's staff see Maternity's patients on their patient list. Maternity nurses see them by role.
Everything else	Decided by role. For example every Laboratory Scientist sees the laboratory queue.

6.2 Examples

Account	What they can do
Doctor + General Medicine	Normal doctor work: their patients, consultations, notes, referrals, prescriptions. No Procedure menu.
Doctor + Procedure	All normal doctor work plus the Procedures station: see, claim, perform and document procedures.
Nurse + Maternity	Triage and nursing work, plus Maternity's patients on the patient list.
Nurse + Procedure	Triage and nursing work plus the Procedures station.
Nurse + Procedure + Theatre	Procedure work through the Procedure membership. Theatre membership is held separately and does not add to or take away from it.
Cashier + Procedure	Cashier work only. A cashier gains nothing from being on Procedure's staff list, because Procedure work needs the Doctor or Nurse role.
Administrator	Every screen. Administrators can open any department's work.

7. Basic navigation

- **Top bar** — the NMHS name (returns to the Dashboard), the **bell** with your unread notification count, your name and role, and **Sign out**.
- **Left menu** — grouped into Workspace, Clinical, Departments, Finance, Pharmacy and Administration. A group only appears if you can use something in it.
- **On a phone or small tablet** the menu is behind the ☰ button at the top left.
- **Notifications** — the bell refreshes about every 30 seconds. Opening a notification takes you to the page it is about. Notifications can be archived, never deleted.
- **Patient picker** — wherever you choose a patient, click the box to browse or type part of a name, hospital number or phone number to filter.

7.1 Menu items by role

Menu item	Shown to
Dashboard, Notifications	Everyone
Patients	Reception, doctors, nurses, maternity nurses, pharmacists, laboratory, radiology, optometrists, eye doctors, cashiers, accountants, ward managers, admins
My Queue	Reception, doctors, nurses, maternity nurses, laboratory, radiology, optometrists, eye doctors, admins
Triage, Send to Doctor	Nurses, admins
Refer Patient	Doctors, eye doctors, admins
Maternity	Doctors, nurses, maternity nurses, reception, admins

Menu item	Shown to
Appointments	Reception, doctors, admins
Laboratory, Lab Catalogue	Laboratory scientists, admins
Ultrasound	Radiology staff, admins
Eye Clinic	Optometrists, eye doctors, admins
Procedures	Doctors and nurses on the Procedure department's staff , admins
Admissions	Ward managers, doctors, nurses, eye doctors, admins
Discharge Patient, Discharged Patients	Administrators
Billing, Outstanding, Waived & Written Off, Transaction History	Cashiers, accountants, reception, admins
Financial Report, Refunds, Service Cancellations, Billing Catalog	Cashiers, accountants, admins
Pharmacy items (Dashboard, Prescriptions, Dispensing, Products, Stock, Stock Count)	Pharmacists, admins
Pharmacy POS	Pharmacists, cashiers, admins
Sales, Returns & Refunds (POS)	Pharmacists, cashiers, accountants, admins
Administration, Departments, Users	Administrators
Inventory, Stock Import / Export	Inventory managers, admins
Expired Items	Inventory managers, admins

8. Dashboard overview

The Dashboard (/) is the first page after sign-in. It shows **cards** with live counts for your role, **alerts** and your recent notifications. Every card opens the page that holds what it counts.

Role	What the dashboard shows
Doctor	My patients, My queue, unread notifications, patients seen today.
Nurse	Waiting for vitals, in progress, vitals and notes recorded today, unread; an alert when someone has waited longer than the vitals-wait threshold (default 30 min).
Cashier / accountant	Taken today, cash today, taken at this desk, part-paid and unpaid counts, outstanding balance, discounted / waived / refunded today; an alert for charges still unpaid after the configured number of hours.
Maternity nurse	Active pregnancies, in labour now, admitted in labour, deliveries today, babies this week, postpartum today; Maternity department and Assigned to me.
Pharmacist	Stock alerts on the dashboard. The Pharmacy Dashboard menu item opens the pharmacy overview: prescriptions to dispense, POS sales today and units on the pharmacy shelf.
Other roles	My queue (where the role receives work), unread notifications and recent activity.

9. Front desk: patients and reception

9.1 Key ideas

Term	Meaning
New patient	Someone never registered at NMHS. They are registered once, for life, and receive a hospital number such as NMHS-P000123.
Returning patient	Already registered. Never register them again — find them with the patient picker or the Patients list.
Consultation	Seeing a doctor. Raised by routing the patient to a doctor (or the Consultation department), or through the Appointments queue.
Department / service workflow	Work done by a unit (laboratory, ultrasound, eye clinic, Procedure, maternity, pharmacy). Usually started by a doctor's referral or prescription; reception routes for vitals, consultation and the clinics it is offered.

9.2 Registering a new patient

Who: Reception (and administrators). **Menu:** Patients → **Register patient** (page </patients/new>).

1. First search the Patients list for the person's name and phone number to be sure they are not already registered.
2. Open **Patients** and press **Register patient**.
3. Enter **First name**, **Last name** (and **Middle name** if any), **Sex**, and either the **Date of birth** or the **Age in years** with its unit.
4. Enter contact details: **Phone number**, optional **Email**, **Street address**, **City**, **State / Region**, **Country**.
5. Enter the emergency contact: **Full name**, **Relationship**, **Phone number**, optionally **Another number** and **Address**.
6. Optionally add a **Short note** (shown under the patient's name on the chart).
7. Save. A confirmation shows the new **hospital number** and a **Print patient card** button.
8. Print the patient card and hand it to the patient.

NOTE: Hospital numbers

The hospital number (NMHS-P...) is issued automatically, is never reused and cannot be edited. Use it on paper and on the phone. Staff numbers use a different prefix (NMHS-S...) so the two can never be confused.

9.3 Finding a returning patient

1. Open **Patients**. The list is alphabetical with a live search box and letter-jump navigation.
2. Type part of the name, the hospital number or the phone number.
3. Click the patient to open their record. Reception sees registration details only; clinical tabs are for clinical roles.

9.4 Billing a card or consultation fee

Who: Reception, cashiers, accountants, admins. **Menu:** Billing.

1. Open **Billing** and pick the patient.
2. In the service picker, choose the category tab (for example **Card Types** or **Consultation Fees**) and tick each service to bill. A chip appears for each choice and a running total is shown.

3. Press **Bill #...** The server prices each service from the Billing Catalog; nobody types the price.
4. Settle it straight away with **Pay in full**, **Pay half**, or **Pay later** (see section 17). Choose the payment method: Cash, Card, Transfer or Insurance.
5. Print the **Receipt** while the patient is at the desk.

IMPORTANT: Payment does not block clinical care

Apart from pharmacy dispensing (section 15), no department refuses to see a patient because a bill is unpaid. Units see the payment status and the cash desk follows up.

Waived or free consultations

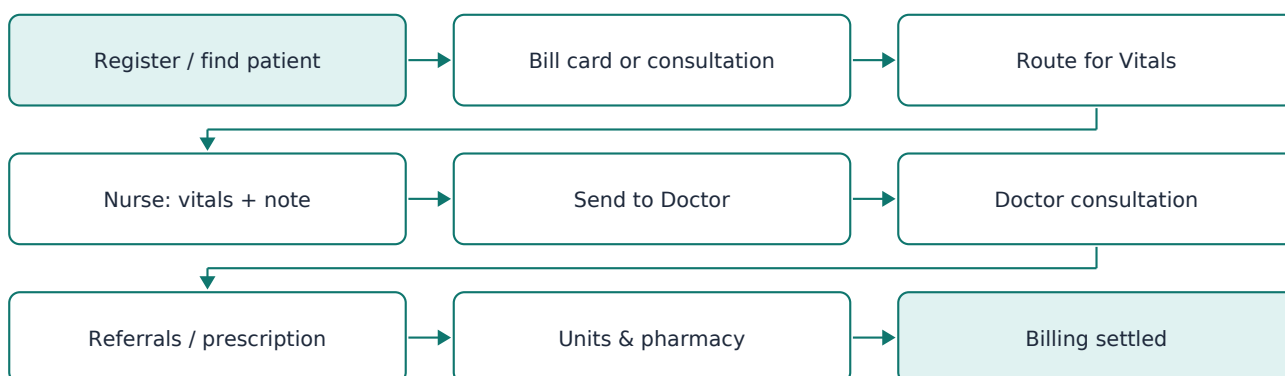
- A **general consultation** queued from Appointments raises **no charge**.
- An already-raised consultation charge can be **waived** (part or all) by a cashier, accountant or administrator, with a mandatory reason. Reception cannot waive.
- Reception **can** authorise **Pay later**, which lets the patient proceed while the money stays owed.

9.5 Routing a patient (the queue)

Who: Reception. **Menu:** My Queue → **Route a patient**.

1. Open **My Queue**.
2. In **Route a patient**, choose the **Patient**.
3. Choose the **Visit type**: Outpatient, Inpatient or Emergency.
4. Choose **Route to department** and **Send for** (the purpose): for example *Vitals (nursing)* → Clinicals (Nursing), *Consultation*, *Ophthalmology / Eye clinic*, or *Maternity*. Picking one usually pre-selects the other.
5. Optionally name the person to receive it. The system refuses a person who cannot do that work (for example a cashier on an eye referral).
6. Set **Priority** (Routine, Urgent, Emergency), the **Reason** and any **Notes**.
7. Submit. Everyone eligible for that work is notified; the patient appears on their queue.

Reception's queue shows only routes raised by the front desk. Reception may **cancel** a route (the patient left, or it was a mistake) but never marks clinical work as started or done.



9.6 Patient record navigation

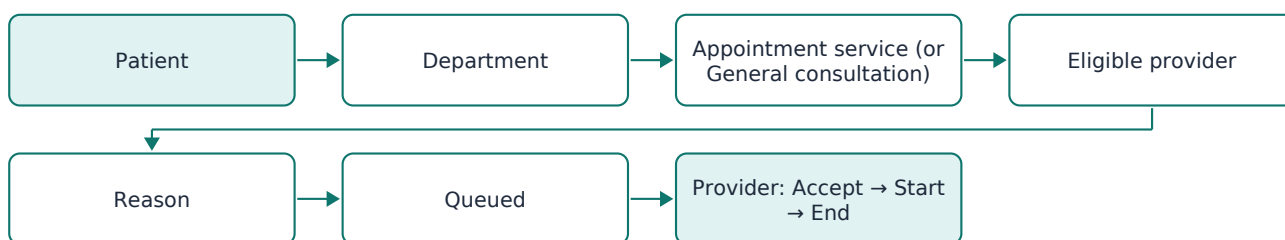
Opening a patient shows an identity band (name, hospital number, sex, age, date of birth, short note), a **Print** button and the tabs your role may see — see section 22.

10. Appointments

IMPORTANT: A queue, not a diary

The HMIS appointment system is a **queue**. Reception chooses the patient, department, service and provider — **there is no date or time picker**. The provider decides when to see the patient; pressing **Start** and **End** records the actual times.

Who: Reception and doctors (and admins). **Menu:** Appointments.



10.1 Queuing an appointment

1. Open **Appointments** and go to **Queue an appointment**.
2. Choose the **Patient** (search by name, hospital number or phone).
3. Choose the **Department**. Only departments an administrator has marked *Available for appointments* are listed.
4. Choose the **Appointment service**, or **General consultation — no charge**. Several services can be booked in one appointment if they belong to the same department.
5. Choose the provider from the list of **eligible** staff (people with the role that work needs, or on that department's staff). The list shows nobody if no eligible provider exists.
6. Optionally enter a **Reason**, then submit. The provider is notified.

10.2 What happens next

- If the service has a price, **one charge per service** is raised immediately at the catalogue price. The appointment shows the fee and the payment state separately. Booking takes no money; payment is taken at Billing. A zero-price service raises no charge.
- The provider opens **Appointments** and presses **Accept**, then **Start** when the patient is seen and **End** when finished. Start and End times are stamped automatically.
- A patient can have only **one open appointment per provider**; a second is refused.
- **Cancelling an appointment does not cancel its charge**. If the service was not received, the bill is withdrawn on the **Service Cancellations** desk (section 17).

10.3 Configuring what can be booked (administrators)

- Department → **Available for appointments** (Django Admin, Departments list).
- Service → **is appointment service** ticked on the billing item. Consultation, eye and ultrasound services were ticked by the initial setup.
- Closing a department for appointments affects only new bookings.

11. Nursing and triage

Who: Nurses. **Menu:** **Triage** (page /vitals). The station has two tabs: **My queue** (live work, filter All / Waiting / In progress) and **Recorded today**.

11.1 Taking vitals

1. Open **Triage**. Patients routed for vitals appear, sorted by priority then waiting time. Unclaimed work shows to every nurse.
2. Press **Accept** on a patient. This claims the patient: they leave the other nurses' queues.
3. Open the patient and record the vitals. The form has units and range checks: **Temperature, Heart rate, Respiratory rate, SpO₂**, blood pressure (**Systolic, Diastolic, Position, Extremity**), **Height, Weight, Glucose** (with when it was **Taken** and whether **Fasting**) and the **Time taken**.
4. Write the nursing note in the same form: **Presenting complaint** (what the patient says is wrong) and your observations.
5. Save. Vitals and nursing notes **lock on save** — they can never be edited. A mistake is corrected by recording a new reading.

11.2 Handing the patient to a doctor

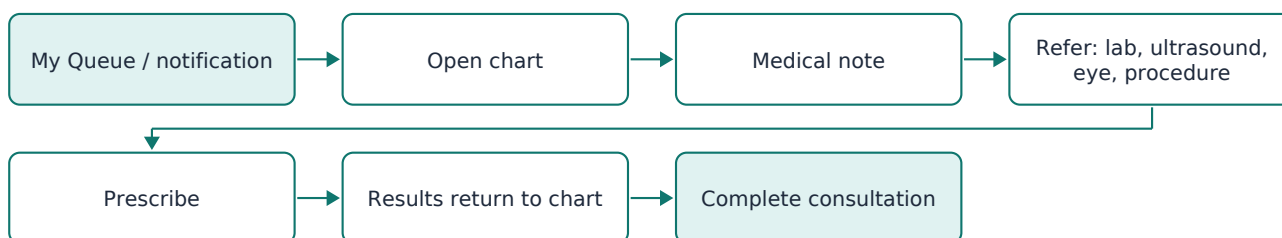
From the patient on the station, or from **Send to Doctor** in the menu:

1. Pick the patient (your current queue, or search the patients routed to you).
2. Choose the doctor (**Select doctor**), the **Priority** and write a **Handover note**.
3. Send. Your vitals route closes, a consultation route opens in that doctor's name, they become the attending doctor and are notified (*Queued for consultation*).

Warning	Meaning and what to do
No vitals recorded	You can still send (a patient may be too distressed for a reading). Confirm, and <i>No vitals recorded for this visit</i> is shown to the doctor.
Reassign	Another doctor already holds the patient. Confirming cancels the old consultation, moves the patient to your chosen doctor and tells the previous doctor.
Mark done blocked	A vitals route cannot be marked done without a reading. Use cancel if the patient left.

Doctors see the reading on the chart's **Vitals** tab and are notified when new vitals land on a patient they hold. A nurse reads back only their own readings; **Recorded today** lists each reading, its note, and which doctor the patient went to.

12. Doctor workflow



12.1 Starting the consultation

1. Open **My Queue** (or the notification *Queued for consultation*). Press **Start** when you begin seeing the patient.
2. Click the patient to open the chart. The **Overview** tab shows allergy alerts, latest vitals, nursing and consultation notes, prescriptions, appointments, routing and all nine health-record sections.

12.2 Writing the medical note

1. Open the **Medical Notes** tab and add a new note.
2. Complete **Reason for visit**, **Chief complaint**, **Notes**, **Diagnosis** and **Plan**. Eye doctors also complete the structured **eye examination**.
3. Save. Each visit is a new note; earlier notes are never overwritten.

NOTE: Correcting a saved note

Press **Amend note** on your own note, choose a **Reason for amendment** and add an explanation. The previous version is archived and the note is marked *Amended*, with its history visible. Only the author or an administrator may amend; colleagues may read.

12.3 Health record

The **Health Record** tab holds nine sections: Allergies, Medications, Medical Conditions, Medical Devices, Surgical History, Family Medical History, Social History, Vaccinations, and Medical Tests & Diagnostics. Each has **Add**, click-to-edit and a delete icon. A medical test can be an uploaded document, typed findings, or both.

12.4 Referring to a department

Menu: Refer Patient (or from the chart).

1. Choose the patient and the destination: **Laboratory**, **Radiology / Ultrasound**, **Ophthalmology / Eye clinic** or **Procedure**.
2. For Laboratory, search the catalogue (FBC, MP, urinalysis...) and pick the tests. For Ultrasound, pick the examinations. For Procedure, pick the procedures from the price list.
3. Set **Priority** and write a **Clinical note**. Optionally name the person to do it.
4. Send referral. The consultation stays open — a referral is work done during it.

Destination	Charge raised when you refer?	Who receives it
Laboratory	Yes — one charge per test, at the order-time price.	Laboratory scientists (shared queue); results come back to the reporting doctor.
Radiology / Ultrasound	Yes — one charge per examination.	Radiology staff.
Eye clinic	No.	Optometrists and eye doctors.
Procedure	Yes — one charge per procedure named.	Doctors and nurses on the Procedure department's staff.

If nobody is posted to receive the work, the referral is still queued and the screen tells you who to ask.

12.5 Prescribing

1. On the chart press **Prescribe**.
2. Search the drug catalogue and add each drug. You see whether a drug is **available** at the pharmacy, never the quantity on the shelf. Out-of-stock drugs are shown but cannot be picked.
3. For each line enter the quantity and directions: dose, frequency, duration, route and notes.
4. Submit the whole script at once. **Each line raises its own charge** at the pharmacy's price and the pharmacy is notified once for the script.

12.6 Completing the consultation

Results appear on the chart tabs (**Lab, Ultrasound, Eye, Procedures**) and you are notified. Close the consultation from **My Queue** with **Mark done**.

13. Laboratory

Who: Laboratory scientists (administrators can also open it). **Menu:** **Laboratory** and **Lab Catalogue**.

13.1 Receiving and working a request

1. Open **Laboratory**. Requests from doctors' referrals appear in the queue with patient context (age, sex, reason, who referred) and their payment status.
2. Press **Accept** to claim it. The request stays visible to the rest of the bench as *Accepted · your name*.
3. Open the order. Tests requested by the doctor are listed; the bench can add a test if needed (it is marked as added by the laboratory and billed the same way).
4. Print the **Request form** to travel with the specimen if required.
5. Enter results. Fill in **only what was measured**; no field is mandatory. Values are text so you can write "< 5", "Nil", "3+" or "Trace". Numbers are flagged low / normal / high against the reference range as you type.
6. Save a draft or **Submit**. Submitting notifies the doctor the result is addressed to (the referring doctor by default; you can redirect it if they are off).
7. **Verify** to release. Verifying closes the referral, files the result on the patient's record and marks it as the final report.

IMPORTANT: Provisional results and corrections

An unverified result can be seen by the doctor but is labelled **provisional** on screen and in print. Changing a **released** result requires a reason and is recorded as an amendment with your name.

13.2 Payment

Ordering a test raises its charge immediately. The bench sees each test's status — Paid, Part paid, UNPAID, Pay later — but **never has to wait for payment**. A sample already drawn is run and the cash desk follows up.

13.3 Laboratory catalogue

Lab Catalogue holds every test, its parameters, units and reference ranges. Adding a test is data entry, not a software change. Tests and parameters are **retired** and can be **restored**; they are never deleted. Changing a price or range does not alter past orders, which keep the values they were taken with.

13.4 Where results appear

- The chart's **Lab** tab (doctors): every order and value with units, the range as it stood when run, flags, amendments and a printable **Laboratory report**.
- Medical Tests & Diagnostics** in the health record, permanently.

14. Ultrasound / Imaging and Eye Clinic

14.1 Ultrasound / Imaging

Who: Radiology staff. **Menu:** **Ultrasound**.

- Open **Ultrasound**. Requests show the examinations ordered and their payment state.
- Press **Accept** to claim. Colleagues see *Accepted · your name*.
- Open the request and write the report in its sections: **Technique, Findings, Measurements, Impression**. You may also upload a report file.
- Press **Save & mark done**. The doctor who asked is notified; the report appears on the chart's **Ultrasound** tab and in the health record.
- Print the **Request form** while the work is open, or the **Report** once written.

14.2 Eye Clinic

Who: Optometrists and eye doctors. **Menu:** **Eye Clinic**. Reception can route patients here directly; doctors can refer. Eye referrals raise no charge.

- Accept the referral. An eye doctor holding a referral sees **Open chart** and **Eye consultation**.
- The eye examination (complaint, ocular history, visual acuity, refraction, pupils, pressures, anterior and posterior segment findings...) is recorded inside the medical note, so it locks and is amended with the note.
- Eye doctors may prescribe, refer onwards and admit their own patients.

15. Pharmacy

15.1 Two different workflows

	Prescription dispensing	Walk-in pharmacy POS
Started by	A doctor's prescription	A customer at the counter
Menu	Pharmacy → Prescriptions	Pharmacy POS
Billing	Each line is billed when the doctor writes it	One payment per sale at the till
Payment gate	Yes — a line is dispensed only when its own bill allows it	Paid at the till before completion
Customer	Always a registered patient	Walk-in (no registration) or registered patient
Stock	From the Pharmacy shelf, earliest expiry first (FEFO)	Same shelf, same FEFO

15.2 Dispensing a prescription

Who: Pharmacists. **Menu:** Pharmacy → **Prescriptions**. Tabs: Overview, Prescriptions, Dispensed & payment, Pharmacy payments, Products, Pharmacy stock, Stock count, Movement history.

1. Open **Prescriptions**. Each line shows the drug, directions and **its own bill**.
2. If the line is unpaid, take payment for **that line** with its payment panel (or send the patient to the cash desk). The money settles that line only, never an older bill.
3. When the line shows PAID, NO PAYMENT REQUIRED or PAY LATER, press **Dispense**. Stock is taken from the Pharmacy shelf, earliest expiry first, and every batch used is logged.
4. Print the **Dispensing note** (or the **Prescription**) for the patient.

Line status	Can it be dispensed?
PAID	Yes
NO PAYMENT REQUIRED (fully waived / discounted)	Yes
PAY LATER (authorised deferral)	Yes
UNPAID or PARTIALLY PAID without a deferral	No — <i>payment required</i>
CANCELLED	No
Not billed (old script)	No — billing roles or the pharmacist can raise its bill first

NOTE: Not enough stock

If the shelf no longer holds enough (for example another pharmacist just dispensed the last box), the line is refused whole, nothing moves, and it stays in the queue to be filled after restocking. Part-fills are not supported.

When a script's bill is settled, the pharmacists are notified *Ready to dispense*. Cancelling a script withdraws its unpaid bill; a paid one is sent to the Service Cancellations desk to be cancelled and refunded.

15.3 Walk-in sales on the POS

Who: Pharmacists and cashiers (admins too). **Menu:** Pharmacy POS.

1. Open the register: enter the **Opening float (cash in the drawer)**.
2. Find products with **Search products** or the category chips and add them to the cart. Change quantities in the cart; **Remove** a line if needed.
3. Optionally choose the **Customer** (a registered patient) or enter **Customer name** and **Customer phone**. A walk-in customer needs no registration.
4. Press **Take Charge**. Choose the **Payment method**: Cash, Card / POS terminal, Bank transfer or Insurance. For cash, enter **Cash received** and the change is calculated.
5. Complete the sale and print or hand over the **receipt**.
6. Use **Hold** to park a sale and **Held sales** → **Resume** to continue it; **Clear** empties the cart.
7. At the end of the shift press **Close register** and enter **Cash counted in the drawer**. The register reports expected cash (float + cash sales – cash refunds) and any variance.

Discounts

- Tick one or more cart lines and press **Discount**, or apply a sale-wide discount. Choose percentage or amount, a **Reason** (mandatory) and confirm. Product prices never change.

- **Who may discount:** cashiers and accountants (and admins), plus any member of staff an administrator has individually authorised. Pharmacists cannot discount unless authorised.
- Above the **cashier's limit** set in Hospital Settings, a supervisor must authorise by typing their own username and password (**Authorising staff username**). Accountants and administrators may approve; a cashier cannot approve their own.
- Above the **maximum**, nobody can approve the discount.

Returns

Returns are taken against a completed sale from **Sales → Returns & Refunds** by a **cashier** (or admin). Returned medicine goes to the *returns-quarantine* location and cannot be resold until stock control releases it; the money goes back as a refund of what was paid.

16. Inventory



16.1 Concepts

Term	Meaning
Product (item)	A catalogue entry: name, category, unit of measure, strength, dosage form, reorder level.
Batch (lot)	A delivery of a product: batch number, expiry date, cost price, sale price, supplier.
Location	Where stock stands. Main Store (receives deliveries) and Pharmacy (the dispensing shelf) are set up; returns-quarantine holds returned POS goods.
Stock record	The quantity of one batch at one location. There is no other stock figure.
FEFO	First-Expiry-First-Out: dispensing and sales always take the earliest-expiring batch on that shelf first. Expired batches are never used.

16.2 Who does what

Action	Who
Create or edit products, categories, units, locations; archive or delete a product	Administrators only (Administration → Products, or Inventory → Add item)
Receive stock, transfer, apply counts, write off, mark a batch expired	Inventory managers and administrators
View stock, batches and movements	Pharmacists, inventory managers, administrators
Count the pharmacy shelf: export the sheet, upload and preview	Pharmacists (their shelf), inventory managers and admins (any location)
Apply an uploaded count	Inventory managers and administrators

16.3 Receiving stock

1. Open **Inventory → Receive stock**.
2. Choose the **Medical item** (searchable), enter the **Batch number**, **Expiry date**, **Quantity**, **Cost price**, **Sale price** and **Supplier**.
3. Check **Receive into** (Main Store by default) and save. The receipt is logged as a movement. An archived product cannot be received.

16.4 Transferring to the Pharmacy

1. Open **Inventory** → **Transfer**. Choose **From** (Main Store) and **To** (Pharmacy).
2. Pick the batches and quantities to move, then confirm. One transfer document with a TRF-reference is created; the movement is logged at both ends.

16.5 Stock counts

- **Physical count** (inventory managers / admins): choose the **Location being counted**, enter counted quantities; the system posts the difference per line as a CNT- document.
- **Import / export** (spreadsheet): export the count sheet as CSV, fill in the counted column, upload it to get a **preview** (nothing moves), then an inventory manager or administrator **applies** it. A file with any problem applies nothing.

16.6 Expired items

Expired Items lists batches past their expiry date or marked expired by hand. **Mark a batch expired** (reason required) takes a lot out of use without deleting anything; it can be returned to use with a reason. Expired stock is cleared with a write-off. Products are archived, not deleted, once they have history.

16.7 Movement history

Every quantity change — receipt, transfer, dispense, sale, return, count adjustment, write-off — is a movement row with who, when, why and the reference. View it in **Inventory** → **Movement log** or **Pharmacy** → **Movement history**.

17. Billing, cashier and finance

17.1 Where charges come from

Source	When the charge is raised	Department credited
Card / consultation (counter)	When billed at the Billing counter	Reception / Consultation
Laboratory test	When the doctor orders it	Laboratory
Ultrasound examination	When the doctor orders it	Radiology
Procedure	When the doctor orders it	Procedure
Prescription line	When the doctor prescribes	Pharmacy
POS sale (registered patient)	At the till	Pharmacy
Appointment service	When the appointment is queued	The service's department
Maternity services	Billed from the Maternity price list	Maternity
Other (write-in)	Typed at the counter with an amount	Unclassified

Prices come from the **Billing Catalog** (cashiers, accountants and admins can edit it). The amount on an existing charge never changes when a price is edited.

17.2 Taking a payment

1. Open **Billing** and pick the patient. Open charges show what is left to pay, what was paid and what was discounted.
2. Use **Pay in full**, **Pay half**, or enter an amount (the quarter / half / three-quarters / everything-owed buttons show the actual figures).
3. Choose the method (Cash, Card, Transfer, Insurance) and confirm.
4. Money is applied to the **oldest open charges first**, unless the payment is taken for a specific bill (as the pharmacy does for a prescription line).
5. Print the **Receipt**: it shows the amount in words, method, who received it and the balance left.

17.3 Who can do what

Action	Reception	Cashier	Accountant	Pharmacist	Admin
Bill services / take payment	Yes	Yes	Yes	Pharmacy bills only	Yes
Pay later (authorise deferral)	Yes	Yes	Yes	No	Yes
Discount (% or amount) / waive	No	Yes	Yes	No	Yes
Refund a payment	No	Yes	Yes	No	Yes
Cancel a service	No	Yes	Yes	No	Yes
Financial report	No	Yes	Yes	No	Yes
Edit Billing Catalog prices	No	Yes	Yes	No	Yes
Transaction history / outstanding / write-off register	Yes	Yes	Yes	No	Yes

17.4 Part payments, pay later, discounts and waivers

Situation	What happens
Part payment	The charge shows PARTIALLY PAID with the remaining balance. Units still work; the pharmacy will not dispense that line until settled or deferred.
Pay later	An authorisation, not a payment: who allowed it, how much and why are recorded. The balance stays owed and appears on Outstanding ; the next payment closes it.
Discount	A percentage or flat amount with a mandatory reason, capped at what is still owed. The original price stays on record; the discount is listed separately.
Waiver	Part or all of the remaining balance forgiven, with a reason and approver. Fully waived services show NO PAYMENT REQUIRED to the units.

Outstanding = original – discount – waiver – payments. None of these figures is ever overwritten; all four stay visible.

17.5 Refunds and service cancellations

Desk	Use it when	Result
Refunds	Money must go back but the service was delivered (overpayment, goodwill).	Refund all or part of a payment. The bill stays active and becomes owed again.
Service Cancellations	The patient never received the service (test not run, procedure not done).	Cancel ends the bill if nothing was paid; Cancel & refund ends it and returns everything paid for it, in one step.

- A reason is mandatory. The original payment is never edited or deleted.

- Services that a department withdrew but that are still billed are pinned at the top of Service Cancellations.
- POS sales are refunded through POS returns, not the Refunds desk.

17.6 Registers and reports

- **Outstanding** — everyone who owes money, biggest first, with phone numbers.
- **Waived & Written Off** — every discount, waiver and refund with reason and approver.
- **Transaction History** — one patient's statement: charges, payments, discounts, waivers and refunds with a running balance. Printable as a **Statement**.
- **Financial Report** — any period (Today, Yesterday, This week, This month, Last month, This year or a custom range). **Collections** (money received; this is revenue) and **Charges** (bills raised and what became of them) are shown separately. Revenue per department, payment-method breakdown, refunds, POS sales and Total Facility Revenue (all payments received minus all refunds).

NOTE: Discounts and waivers are not revenue

Revenue is money received. A discount or waiver reduces what is owed but adds nothing to revenue.

18. Maternity

Who: Maternity nurses (midwives), plus doctors and nurses on the Maternity department's staff; reception for lookup and assignment. **Menu:** **Maternity**.

NOTE: Patient ≠ Pregnancy ≠ Encounter

A woman is registered **once**. A **pregnancy** is an episode (one active at a time). An **encounter** is one attendance within that pregnancy. A returning mother gets a new encounter — never a new registration or a new pregnancy.

18.1 At the maternity desk (reception)

1. Open **Maternity** and look the woman up. The system answers whether she is known and whether she has an active pregnancy, and therefore whether to **Continue** or **Start**.
2. Put her in the Maternity department (assignment) and, optionally, name the responsible **Midwife** and **Doctor**. Reception sees no clinical records.

18.2 Pregnancy and ANC

1. Start the pregnancy (only if none is active) with LMP / EDD and gravida / para.
2. For each attendance open an encounter and choose the **Visit type**: booking, ANC follow-up, consultation, labour assessment, emergency, postnatal or follow-up.
3. Use the **Triage** tab to record vitals — the same vitals form the hospital uses.
4. Gestation is calculated from the LMP on the visit date.

18.3 The maternity workspace

Tab	Contents
Overview	Pregnancy, EDD, gestation, gravida/para, ANC visits, current labour, ward / bed, assigned midwife and doctor.
Triage	Vitals and nursing notes for the mother.
ANC	Each antenatal visit with its own findings, in order.
Labour	The labour episode (onset, stage) and labour observations — one row per check (dilation, fetal heart rate...) forming the partogram .
Deliveries	Delivery type, outcome, placenta, blood loss, complications; <i>N newborns</i> .
Postpartum	Postpartum visits (bleeding, advice, family planning).
Newborns	One card per baby: birth order, weight (g), Apgar 1 / 5 / 10, status. Twins and other multiple births are separate babies of one delivery.

- Labour is admitted through the hospital's **Admissions** (ward and bed); the labour record follows bed transfers automatically.
- Records are corrected with **Amend record** and a reason; the history is kept.
- Discharge is the hospital's normal discharge; maternity adds the mother's and newborn's discharge letters.

18.4 Maternity billing

Maternity services are billed from the price list category *Maternity* (set up with ANC Booking Visit, ANC Follow-up Visit, Obstetric Ultrasound, Delivery Package — Normal, Delivery Package — Caesarean Section, Postnatal Visit and Newborn Care; prices are editable). Tests and scans ordered for her are billed by those departments.

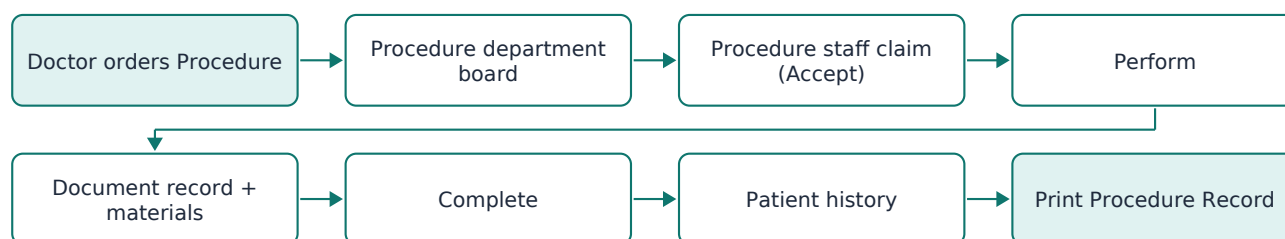
18.5 Maternity documents

Maternity summary, ANC summary, Labour sheet, Partogram, Delivery note (all babies), Birth record (per baby), Postpartum summary, Mother discharge letter and Newborn discharge letter (per baby). Printed by doctors, nurses and maternity nurses.

19. Procedure Department

IMPORTANT: Procedure and Theatre are two separate departments

The **Procedure** department and the **Theatre** department are different departments with different staff. Membership of one gives no access to the other. This section is about **Procedure** only.



19.1 Who are Procedure staff

- A **Doctor** or **Nurse** whose account is on the **Procedure** department's staff list, plus administrators.
- Any number of staff can be on Procedure at the same time, and they may also belong to other departments (including Theatre). Each membership counts only for its own department.
- Other roles (for example a cashier) gain nothing from being on the Procedure staff list.
- Administrators add staff in Django Admin → Departments → **Procedure** (the Staff picker), on the HMIS **Departments** page, or on the user's account (**Users**, authorised departments). It takes effect immediately.
- Only Procedure staff see the **Procedures** menu item. A doctor or nurse who opens the page without being on Procedure's staff is told so, and that an administrator adds staff to 'Procedure' under Departments.

19.2 Ordering a procedure (doctor)

1. Open **Refer Patient**, pick the patient and choose **Procedure**.
2. Under *Which procedure?* pick the procedures from the Procedure price list.
3. Write the clinical note, set priority and send. The referral is filed under the **Procedure** department. One charge per procedure is raised, credited to **Procedure** in the finance reports.
4. Procedure staff are notified. If nobody is posted to Procedure yet, the screen says so.

19.3 Performing and documenting (Procedure staff)

1. Open **Procedures** (the board shows all Procedure work, claimed or not).
2. Press **Accept** to claim and start a procedure. Colleagues see who has it and cannot write on it.
3. Open it and complete the record: **Procedure performed, Procedure notes, Findings, Outcome, Follow-up**.
4. Under **Materials used**, press + **Add material** for each item and fill in **Item / material, Category, Unit, Received, Used, Remaining, Wastage** and **Notes**. Where *Received* is entered, it must equal *Used* + *Remaining* + *Wastage* exactly; the system refuses a row that does not add up and says which row.
5. Press **Save result** to save your work and keep the procedure open, or **Save & mark done** to save and complete it. A procedure cannot be completed without a record.

NOTE: Materials are documentation, not stock

Materials recorded on a procedure do **not** change inventory quantities.

19.4 After completion

- A completed procedure is **locked** for Procedure staff: it opens read-only. Only an administrator can correct it (**Save correction**), and the correction is audited.
- The record appears on the chart's **Procedures** tab and in Medical Tests & Diagnostics; the referring doctor is notified.
- Print the **Procedure Request Form** while the procedure is open, and the **Procedure Record** once it is written — from the Procedures station or the chart's **Procedures** tab. The record prints the sections and the materials as a table.
- Payment is never required before the procedure is performed. When the bill is settled the Procedure team is notified.

20. Theatre

Theatre exists in the current system as a **department** (named *Theatre*) with its own staff list and its own column in the department revenue reports (charges categorised as theatre or surgery).

IMPORTANT: No Theatre workflow is built yet

The current repository contains **no theatre station, theatre booking, operating list or theatre billing category**. The Surgeon and Anesthetist roles exist on the role list but have no dedicated screens. Theatre is **not** the Procedure department and nothing in Procedure is done through Theatre.

21. Inpatient and emergency

21.1 Admissions (bed board)

Who: Ward managers, doctors, nurses; eye doctors for their own patients only. **Menu:** **Admissions** (tabs *On the ward* and *Beds*).

1. Open **Admissions** and choose the ward. Beds show as free or occupied.
2. **Admit a patient:** choose the patient, a free bed and the attending doctor, and enter the **Admitting diagnosis**. A bed can never hold two patients.
3. **Move to** another bed to transfer within or between wards; each move is recorded.
4. **Discharge and free the bed** from the bed board records the discharge and frees the bed.
5. Print the **Admission slip** from the patient's row.

21.2 Admin discharge workspace

Who: Administrators. **Menus:** **Discharge Patient** and **Discharged Patients**.

1. Open **Discharge Patient**, search and select the admission. Review the admission, ward, bed, attending doctor and transfer history.
2. Complete **Discharge diagnosis, Condition on discharge, Summary of the admission, Discharge instructions** and **Follow-up date**.
3. Confirm. A DCH- reference is issued. A second discharge of the same admission is refused and the existing record is shown instead.
4. Print the **Discharge letter** from either screen.

21.3 Emergency

There is **no separate emergency department module**. Emergencies are handled with the visit type **Emergency** and the priority **Emergency** when routing, which places the patient at the top of the receiving queue; maternity has an *Emergency* visit type. Billing and clinical recording are the same as for any other patient.

22. The patient chart and history

Tab	Contents	Who sees it
Overview	Allergy alerts, latest vitals, notes, prescriptions, appointments, routing, all nine health-record sections	Doctors, eye doctors, admins
Health Record	The nine health-record sections	Doctors, eye doctors, admins
Medical Notes	Consultation notes and amendment history	Doctors, eye doctors, admins
Vitals	All readings with who took them	Doctors, eye doctors, nurses, admins
Lab	Orders, values, ranges, flags, reports	Doctors, eye doctors, admins
Ultrasound / Eye / Procedures	Each referral and its report (Procedures includes materials)	Doctors, eye doctors, admins
Admission	Ward and bed, days on ward, attending doctor, stay history	Doctors, eye doctors, admins
Pharmacy	What was dispensed, kept apart from what was only prescribed	Doctors, eye doctors, pharmacists, admins
Billing	Charges, payments, invoice and statement	Reception, cashiers, accountants, admins

Maternity records are read in the **Maternity** workspace. Roles with no chart tab see the patient's registration details only.

23. Printing

All documents print through the browser's own print dialog (choose your printer or **Save as PDF**). They are black on white with the hospital letterhead. The **Print** button prints the document that suits your role and page; others are behind its arrow (caret). A document is only offered when there is a record behind it.

Document	Where	Who can print	Use
Patient card	After registration; chart header	Reception, admins	Give to new patient
Invoice / Statement	Billing counter; chart Billing tab; Transaction History	Billing roles, pharmacist	Open charges / full statement
Receipt	After a payment	Billing roles, pharmacist	Proof of payment
POS receipt	Pharmacy POS after a sale	POS roles	Walk-in or patient sale
Laboratory request form	Lab order header	Lab, clinicians, reception, cashier, accountant	Travels with the specimen
Laboratory report	Lab results panel; chart Lab tab	Lab, clinicians	Result report
Referral request form / Report	Ultrasound, Eye and Procedure stations; chart tabs	Those units, clinicians, nurses (report), reception (form)	Request / finding
Procedure Request Form / Procedure Record	Procedure station; chart Procedures tab	Procedure staff, clinicians	The request; the completed record with materials
Prescription / Dispensing note	Pharmacy counter; chart	Clinicians, pharmacist	Script / what was dispensed
Observation record	Triage station; Vitals tab	Nurses, maternity nurses, clinicians	Vitals history

Document	Where	Who can print	Use
Medical note	Each note's row	Clinicians	One consultation note
Clinical summary	Chart header (doctors)	Clinicians	Referral / handover summary
Admission slip	Bed board row	Ward roles	Admission paperwork
Discharge letter	Discharge screens	Administrators	Given at discharge
Maternity documents (9)	Maternity workspace	Doctors, nurses, maternity nurses	See section 18.5

24. Administration

Administrators (Super Admin and Hospital Admin) work in two places that edit the **same records**: the HMIS **Administration** pages and **Django Admin** (the technical back office). A change in one is seen in the other.

24.1 Staff accounts (Users)

1. Open **Users** and create an account: username, first and last name, email, **role**, **authorised departments** and an initial password.
2. To reset a password, use the account's password reset (**Set password** in Django Admin).
3. To stop someone signing in, deactivate the account rather than deleting it — their records and notifications are kept.

24.2 Departments and staff membership

- Departments are configured on **Departments** (HMIS) or Django Admin → Departments: code, name, manager, **Staff** (many people), active, *Available for appointments*.
- One person may be on several departments' staff lists at once.
- A department that has taken money cannot be deleted — deactivate it instead.
- **Role and department membership are separate**: changing someone's department does not change their role, and vice versa.

24.3 Configuration pages

Page	What it configures
Products, Product categories, Units of measure, Stock locations	The inventory catalogue
Services	Department services
Maternity visit types	Kinds of maternity attendance
Wards, Beds	Inpatient accommodation
Hospital settings	Hospital name, short name, address, phone, email; alert thresholds (expiry warning days, vitals wait minutes, unpaid charge alert hours); POS discount limits, presets and reasons
Notification settings	Switch notification categories on or off (clinical cannot be switched off)
Billing Catalog	Service prices by category (also cashiers and accountants)
Lab Catalogue	Tests and parameters (laboratory and admins)

Rows that history points at cannot be deleted; they are deactivated ("Show inactive" shows them). Django Admin also allows per-user POS discount authorisation (Users → Pharmacy POS) and an

admin view of every notification.

25. Security and good practice

- Access = **role** + **department membership** where a workspace needs it. The server checks every request; hiding a menu is never the security.
- Every clinical and financial action is stamped with the user who did it; many write an audit record.
- Vitals and nursing notes lock on save. Medical notes, laboratory results, procedures and maternity records are corrected by **amendment with a reason**, never overwritten.
- Payments are never edited or deleted; money goes back only through refunds.
- Lock your screen or sign out when you leave a shared computer.
- Report any suspected misuse or access you should not have to an administrator.

26. Troubleshooting

Problem	Check first	Contact an administrator when
Cannot log in	Username spelling and Caps Lock. After five failures wait for the countdown (5 minutes).	You have forgotten the password or the account may be inactive.
Cannot see a menu	Menus follow your role (section 7.1). Procedure needs Procedure membership.	Your role or department membership is wrong.
Cannot see a patient	Search by hospital number or phone. Doctors see the patients they hold; maternity staff see Maternity's patients.	You should be treating the patient but they are not on your list.
Cannot see a department queue	Is the work routed to your unit? Is the status filter set to show it?	Nothing arrives although colleagues receive work.
Cannot claim work	Someone else may have accepted it (<i>Accepted · name</i>).	You need the work reassigned.
Payment problem	Is the charge cancelled, already paid, or deferred? Refresh the page.	A balance looks wrong or a payment did not appear.
Pharmacy will not dispense	Line status must be PAID, NO PAYMENT REQUIRED or PAY LATER. Check stock on the Pharmacy shelf.	Stock is in the Main Store and needs transferring.
Printing problem	Use the Print button's arrow for other documents; check the browser print dialog and printer.	The document is missing for your role.
Stock problem	Is the batch expired or marked expired? Is the stock at the right location?	Counts disagree or a receipt is wrong.
Appointment problem	Is the department open for appointments? Does the patient already have an open appointment with that provider?	No eligible provider is offered.
"Something went wrong" with a reference such as RF-3f2a9c	Try again once.	It repeats — quote the reference so the exact log entry can be found.

27. Quick reference

27.1 Role → main responsibilities

Role	Main responsibilities
Reception	Register, find, route, appointments, bill, pay later, maternity desk
Nurse	Triage vitals, nursing notes, send to doctor, wards, Procedure if posted
Maternity nurse	Pregnancy, ANC, labour, delivery, newborns, postpartum
Doctor	Consult, notes, refer, prescribe, appointments, Procedure if posted
Eye doctor / optometrist	Eye Clinic, eye examination
Laboratory scientist	Lab queue, results, verify, catalogue
Radiology staff	Ultrasound queue and reports
Pharmacist	Dispense, pharmacy payments, POS, count shelf
Cashier / accountant	Payments, discounts, waivers, refunds, cancellations, reports
Inventory manager	Receive, transfer, counts, write-offs, expired items
Administrator	Users, departments, configuration, discharge, everything

27.2 Department → main workflow

Department	Workflow
Reception	Register → bill → route / appointment
Clinicals (Nursing)	Accept → vitals + note → send to doctor
Consultation	Queue → notes → refer / prescribe → done
Laboratory	Order (charged) → accept → results → submit → verify
Radiology	Order (charged) → accept → report → done
Eye	Route / refer → accept → eye examination
Procedure	Order (charged) → accept → record + materials → complete (locked)
Pharmacy	Script (charged per line) → pay line → dispense (FEFO)
Maternity	Lookup → pregnancy → encounters → labour → delivery → postpartum
Theatre	Department and revenue column only; no workflow yet

27.3 Terminology

Term	Meaning
Route / referral	A request sending a patient to a unit's queue
Accept / claim	Taking a piece of unit work as your own
Charge	One billed service on a patient's account
Deferral (pay later)	Authorisation to proceed while money is still owed
Waiver	Forgiving part or all of a charge
Amendment	A correction with a reason; the previous version is kept
FEFO	First-Expiry-First-Out stock use
Hospital number	NMHS-P... (patient) or NMHS-S... (staff)

27.4 Support and escalation

1. Check the troubleshooting table (section 26) and try again once.
2. Ask your supervisor or a colleague in the same role whether they see the same problem.
3. Contact the HMIS administrator with: your username, the patient's hospital number (never the patient's clinical details in an insecure message), the page, what you did, and any reference shown on screen (for example RF-...).
4. For anything affecting patient safety, follow the hospital's clinical escalation procedure first; do not wait for the software.